



NSE: IKS | BSE: 544309
IT-Enabled Services / Healthcare

PERSONAL NOTES

My View | May 2026

Inventurus Knowledge Solutions

The chore-taker becomes the system of record

Raghav | May 2026

		Price (May 2026)	
		₹ 1,887	

Market cap (₹ Cr)	32,622	52-week range (₹)	1,262–1,933	Promoter holding	63.7%
Shares o/s (Cr)	17.3	FY26 revenue (₹ Cr)	3,194	FII / DII	8.2% / 6.4%
Free float (₹ Cr)	11,840	FY26 PAT (₹ Cr)	722	20-day avg volume	1.44 lakh sh.
P/E FY26A / FY28E*	44.9x / 27.2x	Pro-forma net debt (₹ Cr)	~4,900	Dividend yield	Nil

*FY28E on adjusted EPS (excluding acquisition-intangible amortisation). Source: NSE/Groww quote data as at May 2026, company filings, my own analysis.

INVESTMENT SUMMARY

Inventurus Knowledge Solutions (“IKS Health”) sells US doctors and hospitals their own back office. It takes on the administrative, clinical-documentation and revenue-cycle chores of American physician enterprises — scheduling and prior authorisation, ambient AI scribing, medical coding, claims and denials, and increasingly value-based-care analytics — under multi-year contracts priced on outcomes rather than headcount. FY26 revenue was ₹3,194 Cr (+20% YoY) with a 34.2% EBITDA margin and ₹722 Cr of PAT (+48%), on 40% ROE and effectively no net debt.

On 9 July 2026 IKS closed the US\$557mn all-cash acquisition of Nasdaq-listed TruBridge, funded almost entirely with ~US\$600mn of five-year debt. That single transaction changes the investment case more than anything since the IPO: it roughly doubles revenue, adds 1,500+ rural and community hospitals to a client base of ~450 large ambulatory enterprises, and — critically — gives IKS ownership of an *electronic health record*, the system of record inside which its AI agents will run. It also takes the balance sheet from net cash to roughly 2.8x EBITDA and dilutes group margin from 34% to ~29% for two years.

I think fair value sits around ₹2,180 (+15.5%), set at the convergence of a 10-year DCF (₹2,167 at a 10.9% WACC and 5.0% terminal growth), 32x FY28E adjusted EPS of ₹69.4 (₹2,222) and 18x FY28E EBITDA (₹2,188). I am not adding aggressively at today's price because the stock sits within 3% of its all-time high, reported FY27 EPS goes sideways as interest and purchase-price amortisation land, and the thesis now depends on a two-year technology integration the company has never attempted at this scale.

What underwrites the thesis

- **A genuine, measurable moat.** Contracts are embedded in daily clinical workflow, not in a procurement cycle: top-10 client vintage is close to six years, switching means re-training physicians, and IKS holds Black Book's number-one client-rated position in AI-driven RCM, medical coding (four years running) and clinical documentation (thirteen years running).
- **Operating leverage that is already visible.** Operating margin has expanded for eight consecutive quarters, from 20.3% in Q4 FY24 to 34.4% in Q4 FY26, while FY26 headcount grew only 5.3% against 20% revenue growth. This is the clearest evidence available that AI am decoupling revenue from people in this business.

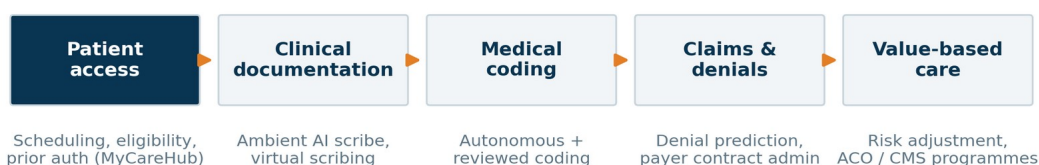
- **A structurally growing, under-penetrated market.** The US revenue-cycle-management market is put at roughly US\$73bn in 2026 growing ~11.6% a year, with services about 77% of it. IKS's FY26 revenue is roughly 0.5% of the services pool it addresses.
- **TruBridge was bought cheaply.** US\$557mn EV against ~US\$347mn revenue and ~US\$69mn adjusted EBITDA is 8.1x EBITDA and 1.6x sales — against IKS's own 27x trailing EV/EBITDA. Even before synergies, the arithmetic favours the acquirer.
- **Management has set a checkable target.** "True North" is ₹3,000 Cr of EBITDA by FY30, roughly triple FY26. My own base case reaches ₹3,333 Cr in FY30E — I am not relying on management optimism to justify the fair value.

WHAT THE BUSINESS ACTUALLY DOES

American healthcare has an administrative problem that is structural rather than cyclical. A US physician generates a document trail that must satisfy a payer before it converts into cash: the visit must be documented to clinical standard, coded to the correct ICD/CPT level, checked for prior authorisation, submitted, and defended when denied. Practices lose an estimated 5–10% of potential revenue to coding inaccuracy and missed charges alone. That work does not scale with technology alone, because the edge cases are clinical judgement calls, and it does not scale with labour alone, because labour costs are what caused the problem.

IKS positions itself between the two. It calls the model "care enablement": a platform of roughly sixteen solutions spanning the physician value chain, delivered by a blend of proprietary AI agents and 13,331 domain-trained staff largely based in India. Pricing is increasingly outcome-linked — the company describes it as fee-for-value and is willing to share risk — which is a materially different commercial posture from the per-FTE billing that defines conventional healthcare BPO.

Exhibit 1: IKS sells the whole administrative chore-stack, not a point tool



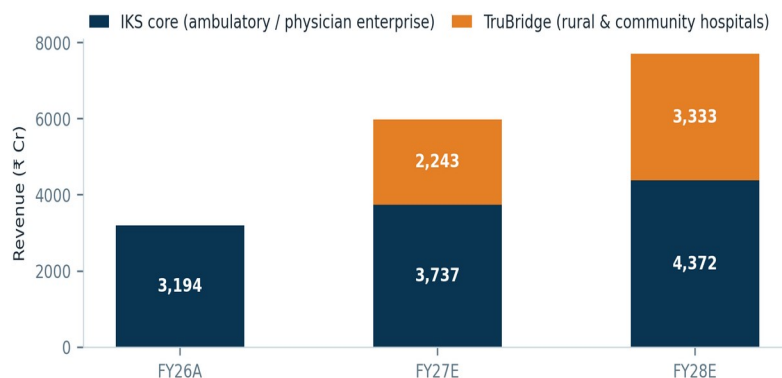
Source: Company website and disclosures, my own analysis. Data as of May 2026.

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Geographically the business is almost entirely US-facing (with small Canada and Australia exposure) and delivered from India, so reported INR growth carries a currency component on top of underlying USD growth — Q4 FY26 revenue grew 18% in rupees but 13% in dollars, a gap worth remembering whenever headline growth is quoted.

From one engine to two

Until July 2026, IKS sold services into ambulatory and physician-enterprise clients: roughly 450 large enterprise relationships including Mass General Brigham, Texas Health Care PLLC and GI Alliance, deliberately pruned down from over 850 clients as management culled small, low-margin accounts in favour of large groups and health systems. TruBridge brings a completely different customer: the small rural and community hospital that cannot afford an in-house IT or billing department, and which buys its EHR and its billing from the same vendor.

Exhibit 2: Two engines from FY27 — a high-margin core plus a scale hospital franchise

Source: Company filings, my own analysis; TruBridge revenue of US\$347mn converted at ₹88/US\$. Data as of FY27E consolidates TruBridge from 10-Jul-2026 (~8.7 months).

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The two businesses have opposite financial characteristics. The IKS core runs at ~35% EBITDA margin and mid-to-high-teens growth. TruBridge runs at roughly 20% margin (US\$69mn on US\$347mn) and low-single-digit growth. Blending them takes group margin down to an estimated 29.3% in FY27E before offshoring, automation and cross-sell begin to pull it back up. Anyone reading FY27 headline margin compression as deterioration is misreading a mix effect.

WHY THE SECTOR LOOKS GOOD

Three forces are compounding at once, and none of them is a function of the US economic cycle.

1. Administrative complexity is growing faster than healthcare itself

Payer rules, prior-authorisation requirements, Medicare Advantage risk adjustment, telehealth billing scenarios and value-based-care reporting have each added a layer of documentation obligation on providers over the last decade. Market estimates put the US RCM market at roughly US\$73bn in 2026 heading to ~US\$196bn by 2035 — an 11.6% CAGR — with a parallel estimate of US\$77bn in 2025 growing at 11.7%. Different houses, similar shape: low-teens compounding, driven by complexity rather than volume.

Exhibit 3: IKS is roughly 0.5% of the US RCM services pool it addresses

Source: Towards Healthcare / Mordor Intelligence (market size); company release (rural TAM); company filings. Data as of 2026 estimates; IKS revenue converted at ₹88/US\$.

Source: Towards Healthcare and Mordor Intelligence (market size and services share); company release (rural TAM); company filings. 2026 estimates; IKS revenue converted at ₹88/US\$.

2. Provider economics are deteriorating, which is good for outsourcers

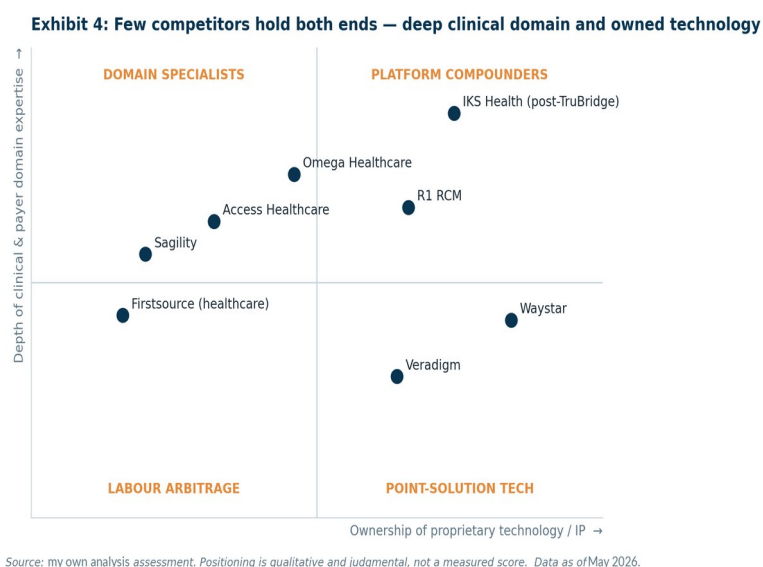
Hospital and physician-group margins have been squeezed by wage inflation, denial rates and reimbursement pressure. When margins are comfortable, administrative inefficiency is tolerated; when they are not, it becomes a board-level line item. Services already account for roughly 77% of the RCM market precisely because providers prefer to hand the problem, and the technology capex behind it, to someone else. Rural and community hospitals — TruBridge's base — are the most financially stressed cohort in the system, and therefore the most motivated buyers of a cheaper administrative stack. This cuts both ways: it is also the cohort most exposed to Medicaid funding changes, which I treat as a live risk below.

3. AI has arrived at exactly the layer where this industry works

Ambient documentation, autonomous coding and denial prediction are among the very few genuinely production-grade enterprise AI applications today, because the tasks are high-volume, structured, and have a verifiable ground truth. For an incumbent with clinical data, workflow access and a trained review layer, AI is margin expansion. For a pure labour-arbitrage vendor with none of those, AI is an existential price war. The sector is about to separate along that line, and the sorting is the investment opportunity.

Where IKS sits

The competitive field splits along two axes: how deep a player's clinical and payer domain expertise runs, and how much of the technology it actually owns rather than licenses. Labour-led Indian healthcare BPM (Sagility, Firstsource's healthcare book, Access Healthcare) is strong on the first and weak on the second. US health-tech vendors (Waystar, Veradigm) are the reverse. IKS — now holding an EHR, an agentic platform and 20 years of clinical operating history — occupies the corner where both are true (Exhibit 4).



Source: my own analysis assessment. Positioning is qualitative and judgmental, not a measured score. As of May 2026.

THE MOAT: WHAT IT IS, AND HOW YOU WOULD KNOW

Moat claims are cheap. The useful question is which of them leave a fingerprint in the numbers. I find four that do, and one that does not yet.

1. Workflow embedding and switching costs — evidenced

IKS is not a vendor a client can swap over a quarter. Its people and agents sit inside the client's EHR, its scribes are trained on individual physicians' documentation habits, and its coders are specialty-specific. Replacing it means retraining clinicians and accepting a documented dip in cash collection during transition — in a department where a two-week revenue interruption is a board conversation. The fingerprint: average relationship vintage of nearly six years across the top clients, and revenue growth continuing through a deliberate cull of the client base from 850 to a targeted ~500. A business with weak stickiness cannot shrink its customer count by 40% and grow revenue 20%.

2. A proprietary clinical data corpus — newly and materially strengthened

This is the strategic point of TruBridge and the part most likely to be underestimated. IKS previously ran AI agents inside other vendors' systems of record. It now owns an EHR holding data on more than 15 million patients, which becomes both a training corpus and a distribution rail: agentic workflows can be orchestrated natively inside the record rather than bolted on top. Combined with the May-2026 acquisition of ARAI Solutions — explicitly aimed at reducing dependence on third-party AI models — the company is building toward owning the full stack from data to agent to workflow. Very few competitors can assemble that set.

3. Regulatory and trust barriers — structural

HIPAA compliance, HITRUST certification, SOC attestation and payer-facing audit history are not difficult to describe and are very difficult to accumulate. A US health system will not put patient records through an unproven offshore counterparty, which is why credible entrants in this market are almost always acquired rather than built. Black Book's 2026 client-rated rankings — first in AI-driven RCM, first in medical coding for a fourth consecutive year, first in clinical documentation for a thirteenth — are a proxy for exactly this accumulated trust, and they are voted by clients rather than bought.

4. Scale economics in a fragmented market — emerging

Coding, denial management and prior authorisation all improve with data volume: more claims means better denial prediction, more specialty coverage means better autonomous coding accuracy. The market remains fragmented across hundreds of regional vendors, so each acquisition adds both revenue and model quality. This is the flywheel management is buying, and it is why the acquisition strategy is coherent rather than opportunistic.

The moat I would not yet credit

Pricing power is not proven. Management was asked directly on the Q4 FY26 call about deflation risk — the possibility that AI-driven productivity gets competed away to clients as lower prices rather than retained as margin. The honest answer is that nobody knows yet. Eight quarters of margin expansion argue that IKS is currently keeping the gains; a five-year view has to allow that outcome-linked pricing may cap how much it keeps. I model margin flattening at ~35% rather than continuing to climb, and I treat any outcome above that as upside rather than base case.

Exhibit 5: A serial acquirer that has so far integrated well



Source: Company disclosures, exchange filings, my own analysis. Data as of May 2026.

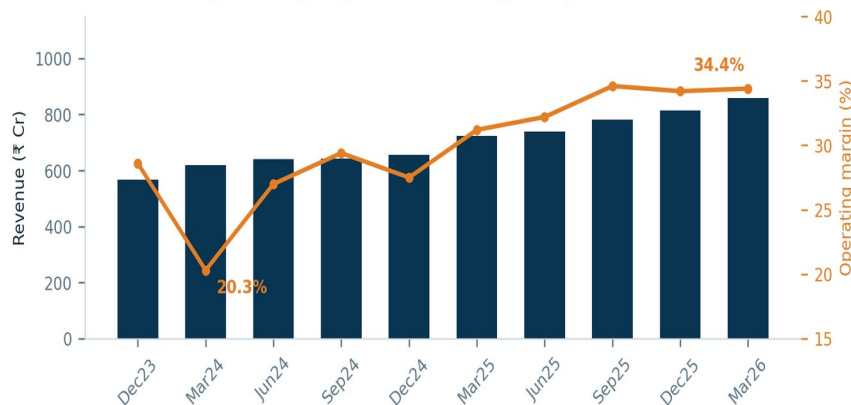
Source: Company disclosures, exchange filings, my own analysis. As of May 2026.

INVESTMENT THESIS

1. The AI margin story is already in the reported numbers, not the guidance

Most "AI beneficiary" cases in Indian IT are prospective. This one has eight quarters of evidence. Operating margin went from 20.3% in Q4 FY24 — the quarter absorbing AQuity integration costs — to 34.4% in Q4 FY26, while FY26 headcount rose only 5.3% against 20% revenue growth. Revenue per employee moved from roughly ₹21 lakh to ₹24 lakh in a single year. That is what non-linearity looks like when it is real (Exhibit 6).

Exhibit 6: Eight straight quarters of margin expansion — 20% to 34% without headcount growth



Source: Company filings, my own analysis. Data as of Q4 FY26 (Mar-2026).

Source: Company filings, my own analysis. Data as of Q4 FY26 (Mar-2026).

2. TruBridge is accretive arithmetic before it is a synergy story

IKS paid 8.1x EBITDA and 1.6x sales for a business with recurring revenue and 1,500+ sticky hospital clients, funded at roughly 7–7.5% debt cost, while its own equity trades at 27x EV/EBITDA. Even holding TruBridge flat and capturing nothing, the deal creates value for IKS shareholders on the multiple gap alone. Management has separately quantified US\$575mn of cross-sell whitespace — layering IKS's coding, documentation and denial tools onto TruBridge's installed base. I include none of that in my base case, which is why my FY30E EBITDA of ₹3,333 Cr sits above management's own ₹3,000 Cr “True North” target without heroic assumptions.

3. Owning the system of record changes the competitive question

Before July, the bear question was “what happens when Epic or the EHR vendors build these agents themselves?” IKS's answer is now to be an EHR vendor in the segment it targets. It plans to migrate TruBridge's legacy platform from COBOL to Postgres over 18–24 months and orchestrate agents natively inside it. If that lands, IKS is selling a combined system of record and system of action to a customer segment that has historically been unable to afford either — and doing so with a US\$260bn stated addressable opportunity in rural and community health.

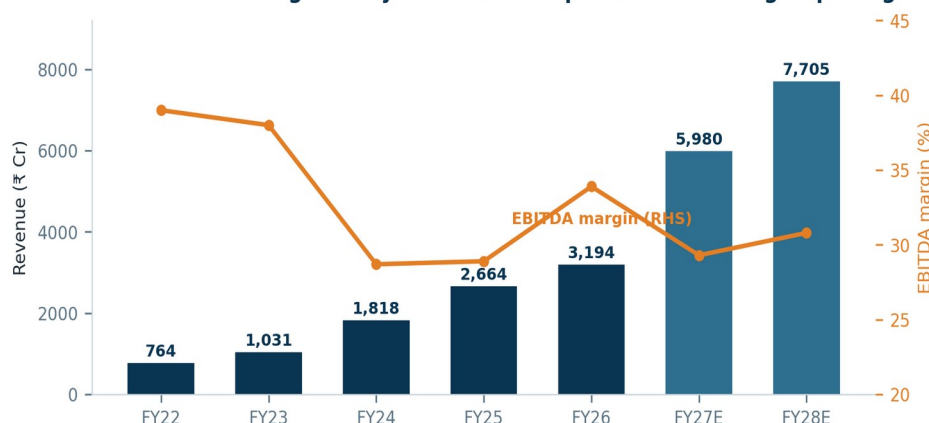
4. Cash generation funds the deleveraging without equity

FY26 free cash flow was ₹613 Cr on operating cash flow of ₹674 Cr — an 80% conversion of operating profit, up from 56% in FY25 — and the company retired net debt from ₹850 Cr to ₹251 Cr during the year before re-levering for TruBridge. Capex intensity is structurally low (FY26 capex was under ₹35 Cr). On my estimates the group generates ₹989 Cr of FCFF in FY27E rising to ₹1,537 Cr in FY28E, which services ~2.8x pro-forma leverage comfortably and takes it below 1.5x by FY29E. CARE assigned an A/Stable rating in June 2026.

5. Ownership is aligned and the float is tight

Promoters hold 63.7% and have not sold since the December 2024 IPO, which was itself entirely an offer for sale. The promoter group includes the Aryaman Jhunjhunwala Discretionary Trust at 16.4%; the board is chaired by Berjis Desai and the business is run by founder Sachin Gupta with CFO Nithya Balasubramanian. Institutional ownership has risen steadily — FII from 5.5% to 8.2% and DII from 4.5% to 6.4% over five quarters — against a free float of roughly ₹11,800 Cr. The corollary is genuine illiquidity risk: 20-day average volume is about 1.4 lakh shares.

Exhibit 7: TruBridge nearly doubles the top line but dilutes group margin for two years



Source: Company filings, my own analysis. Data as of FY26 actuals; FY27E–FY28E my own estimates.

Source: Company filings, my own analysis. FY26 actuals; FY27E–FY28E my own estimates.

FINANCIAL SUMMARY

₹ Cr (consolidated)	FY24A	FY25A	FY26A	FY27E	FY28E
Revenue	1,818	2,664	3,194	5,980	7,705
Growth (%)	76.3	46.5	19.9	87.2	28.8
EBITDA	521	771	1,091	1,754	2,374
EBITDA margin (%)	28.7	28.9	34.2	29.3	30.8
Depreciation & amortisation	59	113	124	560	750

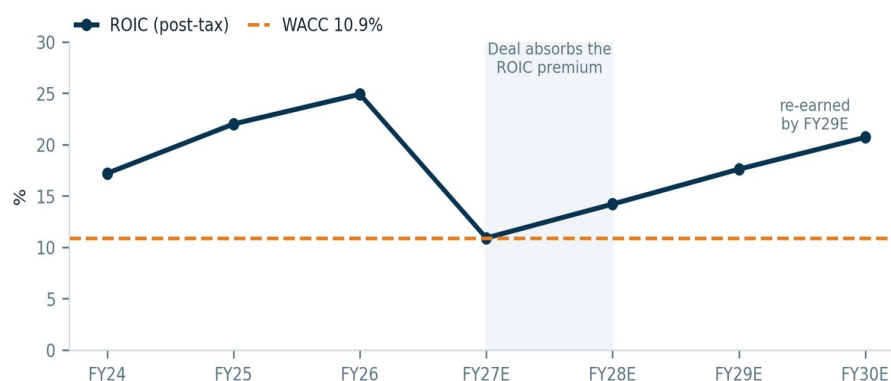
₹ Cr (consolidated)	FY24A	FY25A	FY26A	FY27E	FY28E
EBIT	462	658	967	1,194	1,624
Net interest	61	91	70	310	350
Profit before tax	442	606	905	904	1,299
Reported PAT	370	486	722	705	1,013
Adjusted PAT*	—	551	809	838	1,200
Reported EPS (₹)	21.68	28.33	42.03	40.8	58.6
Adjusted EPS (₹)*	—	32.1	47.1	48.4	69.4
ROE (%)	32.0	27.2	40.3	22.2	24.9
ROIC (% post-tax)	17.2	22.0	24.9	10.9	14.2
Free cash flow	177	276	613	989	1,537
Net debt	1,100	600	251	4,300	3,300
Net debt / EBITDA (x)	2.1	0.8	0.2	2.5	1.4
P/E on reported EPS (x)	87.0	66.6	44.9	46.3	32.2
P/E on adjusted EPS (x)	—	58.8	40.1	38.9	27.2
EV / EBITDA (x)	—	—	30.1	21.4	15.8

*Adjusted PAT and EPS exclude acquisition-related intangible amortisation, consistent with the company's own reporting convention (FY26 adjusted PAT of ₹809 Cr against ₹722 Cr reported). FY24–FY26 from company filings and Screener; FY27E–FY28E my own estimates. Multiples computed on CMP of ₹1,887 and pro-forma net debt. FY24–FY25 net debt is my own estimate from reported borrowings.

Two things in this table matter more than the rest. First, FY27E reported EPS goes sideways — ₹40.8 against ₹42.03 — because roughly ₹310 Cr of interest and ₹170 Cr of purchase-price amortisation arrive before the offsetting operating contribution has had time to compound. On adjusted EPS the picture is different (₹48.4 versus ₹47.1), and it is the adjusted line that reflects cash economics. Investors who anchor on reported FY27 EPS will conclude the deal destroyed value; that conclusion would be premature by about eighteen months.

Second, ROIC. Post-tax ROIC falls from 24.9% in FY26 to roughly 10.9% in FY27E — which is to say, to the cost of capital. This is the honest price of the transaction: IKS has temporarily converted a high-return, low-capital business into an average one. My base case says it re-earns the premium by FY29E (Exhibit 8). If it does not, the valuation case does not hold.

Exhibit 8: The value-creation test — ROIC falls to the cost of capital in FY27E, then recovers



Source: my own estimates; ROIC = NOPAT / (net debt + equity). Data as of May 2026.

Source: my own estimates; ROIC = NOPAT / (net debt + equity). As of Aug-2026.

VALUATION

I triangulate three methods and weight the DCF most heavily, because the entire debate about IKS is about the duration and durability of cash flows rather than about next year's multiple. A discounted cash flow forces those assumptions into the open; a P/E on a transition year hides them.

Assumptions and cost of capital

Input	Value	Basis
Risk-free rate	6.50%	10-year Indian G-Sec, assumed level at valuation date
Equity risk premium	6.00%	House standard for Indian equities
Beta	0.85	Low observed beta on a short (20-month) listed history; see limitations
Cost of equity	11.60%	CAPM: 6.50% + 0.85 × 6.00%
Pre-tax cost of debt	7.30%	Indicative USD term-loan pricing on the ~US\$600mn facility
Post-tax cost of debt	5.62%	At a 23% marginal tax rate
Target debt weight	12%	Steady state after deleveraging (currently ~15%)
WACC	10.88%	Rounded to 10.9% in all outputs
Terminal growth	5.00%	c.3% USD terminal growth plus c.2% assumed INR depreciation
Tax rate	23%	Blended India/US effective rate; FY26 actual was 20.3%
USD/INR	88.0	Assumption for translating TruBridge financials

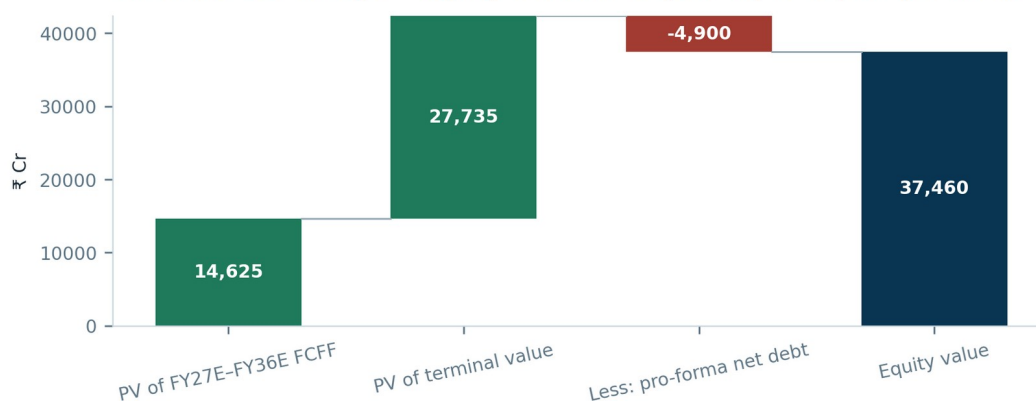
Source: my own analysis. Rows marked as assumptions are my own inputs, not company guidance. Cross-check: a 5.0% Gordon growth rate implies a terminal EV/EBITDA of 12.3x, which is below IKS's current 27x and below the ~16x I apply in the relative cross-check — i.e. the DCF does not smuggle in multiple expansion.

Free cash flow projection

₹ Cr	FY27E	FY28E	FY29E	FY30E	FY31E	FY32E
Revenue	5,980	7,705	8,861	10,101	11,414	12,784
Growth (%)	87.2	28.8	15.0	14.0	13.0	12.0
EBITDA	1,754	2,374	2,835	3,333	3,858	4,385
Margin (%)	29.3	30.8	32.0	33.0	33.8	34.3
Less: D&A	(560)	(750)	(753)	(808)	(856)	(895)
EBIT	1,194	1,624	2,082	2,525	3,002	3,490
NOPAT (23% tax)	919	1,250	1,603	1,944	2,312	2,687
Add: D&A	560	750	753	808	856	895
Less: capex	(239)	(308)	(354)	(384)	(434)	(486)
Less: change in NWC	(251)	(155)	(104)	(112)	(118)	(123)
FCFF	989	1,537	1,898	2,257	2,616	2,973

Source: my own estimates. Explicit forecast period runs FY27E–FY36E (10 years); FY33E–FY36E omitted here for space, with revenue growth fading from 11% to 7.5% and margin held at 35%. Capex assumed at 3.5–4.0% of revenue, reflecting TruBridge's capitalised software spend against IKS's historically minimal capex.

Note the FY30E EBITDA line: ₹3,333 Cr against management's stated "True North" target of ₹3,000 Cr. My model is marginally ahead of the company's own ambition, without any cross-sell synergy assumption. That is a comfortable place to be — it means the fair value does not require management to outperform its own plan.

Exhibit 9: DCF bridge — equity value of ₹37,500 Cr, or ₹2,167 per share

Source: my own analysis. WACC 10.9%, terminal growth 5.0%. Data as of Valuation date May 2026.

Source: my own analysis. WACC 10.9%, terminal growth 5.0%. Valuation date May 2026.

Terminal value is ₹27,735 Cr of present value, or **65.5% of enterprise value**. That is within the normal range for a growth compounder with a 10-year explicit period, but it is the single number a reader should stress first — two-thirds of this valuation sits beyond FY36.

Sensitivity: value per share (₹) at varying WACC and terminal growth

WACC \ terminal g	4.0%	4.5%	5.0%	5.5%	6.0%
9.9%	2,348	2,519	2,725	2,977	3,295
10.4%	2,117	2,255	2,419	2,617	2,859
10.9% (base)	1,920	2,034	2,167	2,324	2,513
11.4%	1,751	1,845	1,954	2,081	2,232
11.9%	1,603	1,682	1,773	1,877	2,000

Source: my own analysis. Base case (10.9% WACC, 5.0% terminal growth) highlighted. Note that the current price of ₹1,887 is discounted by a 11.4% WACC with 4.8% terminal growth — the market is not pricing an aggressive scenario.

Scenarios

Scenario	What differs from base	Fair value (₹)	vs CMP	Weight
Bear	TruBridge integration slips beyond 24 months; AI productivity is competed away into pricing (margins 300bp below base); rural reimbursement pressure caps hospital-segment growth. WACC 11.4%, g 4.5%.	1,440	(24)%	25%
Base	Core grows 17% through FY28E; TruBridge margin lifts 20% → 24% by FY28E on offshoring and automation; no cross-sell synergy credited. WACC 10.9%, g 5.0%.	2,167	+15%	55%
Bull	Cross-sell into the 1,500 TruBridge hospitals converts at even a third of the stated US\$575mn whitespace; EHR migration completes on schedule and margins reach 37%. WACC 10.4%, g 5.5%.	3,220	+71%	20%

Source: my own analysis. Probability-weighted fair value on these weights is ₹2,190, which I round to a ₹2,180 target. Weights are the analyst's judgement.

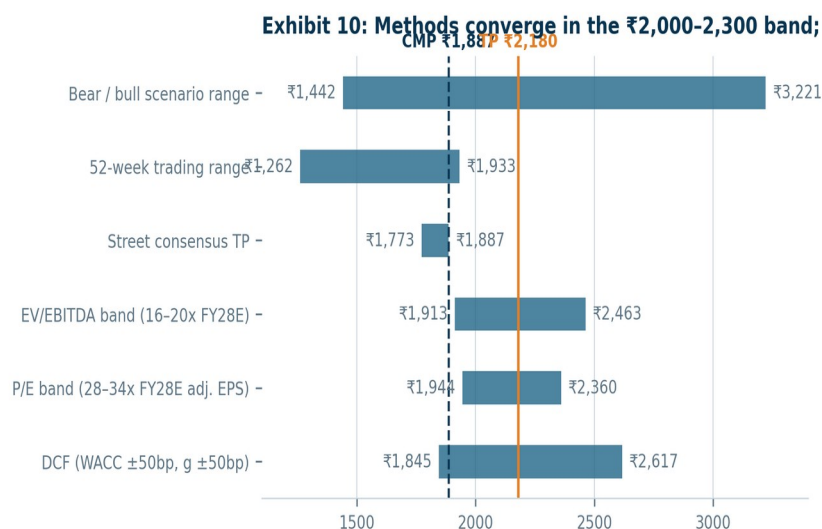
Relative cross-check

Company	Mcap (₹ Cr)	Price (₹)	P/E (x)	P/B (x)	Implied ROE
Inventus Knowledge (IKS)	32,622	1,887	45.2	11.65	25.8%
Sagility India	19,676	41.9	21.2	2.04	9.6%
Firstsource Solutions	18,736	269.9	27.8	4.26	15.3%
eClerx Services	15,638	1,644	22.1	6.10	27.6%
Latent View Analytics	6,283	301.1	31.1	3.58	11.5%
Peer median (ex-IKS)	17,187	—	25.0	3.92	13.4%

Source: Groww/Refinitiv consolidated data as at May 2026. P/E is trailing twelve months. Implied ROE is computed as P/B ÷ P/E from the same source for comparability across rows; it will differ from each company's reported ROE. Peers are the closest listed Indian comparables by delivery model, not by end-market — there is no listed Indian pure-play in US provider RCM.

IKS trades at roughly 1.8x the peer-median P/E. On these numbers that premium is defensible rather than generous: it earns roughly double the peer-median return on equity, grew revenue 20% against a peer set growing in single digits, and is the only name in the list that owns proprietary clinical technology rather than selling process capacity. The multiple is not the risk here; the integration is.

I note also that my fair value sits above the current sell-side consensus, which spans a median of ₹1,774 (LSEG, eight analysts) to a mean of ₹1,887 (StockAnalysis). My gap to consensus is deliberate and comes almost entirely from crediting TruBridge's FY29–FY30 contribution rather than from a higher multiple.



Source: my own analysis; consensus from LSEG (median) and StockAnalysis (mean), 8 analysts. As of May 2026.

RISKS

- **Integration execution — the dominant risk.** TruBridge is the largest acquisition IKS has attempted, and the plan includes re-platforming a legacy EHR from COBOL to Postgres over 18–24 months while cross-selling into its client base. Management has flagged the timeline itself. A slip past 24 months, or client attrition during migration, would push the ROIC recovery beyond FY29E and remove most of the gap between my fair value and the current price. I would revisit my view if TruBridge organic revenue declines in two consecutive quarters, or if group EBITDA margin fails to exceed 30% by Q4 FY28.
- **AI-driven price deflation.** If clients capture the productivity gains rather than IKS, revenue per transaction falls faster than cost. This is the risk management was pressed on at the Q4 FY26 call and could not fully answer. The early warning sign is revenue growth decelerating while volumes hold — watch the gap between USD revenue growth and client/volume additions rather than headline INR growth.
- **Leverage in a rising-rate or weak-dollar scenario.** Pro-forma net debt of roughly ₹4,900 Cr, backed by a US\$603.75mn corporate guarantee, is comfortable at 2.8x EBITDA but leaves less room than the market is used to from this company. The debt is USD-denominated against USD earnings, which is a natural hedge; the exposure is to floating-rate cost, not to currency.
- **US rural reimbursement policy.** TruBridge's clients are rural and community hospitals, the cohort most sensitive to Medicaid and rural-health funding decisions. Adverse funding changes could close client hospitals outright — a form of attrition no commercial execution can offset. Conversely, dedicated rural-health transformation funding would be a direct tailwind. This is the newest and least analysed exposure in the story.
- **Client and geographic concentration.** Essentially all revenue is US provider-side. A single large health-system loss, a US policy shift on offshore handling of patient data, or India-US services trade friction would each hit the whole book at once. The 2026 US tariff environment has not touched services, but the precedent for policy risk to services delivery is now established.
- **Liquidity and float.** A 63.7% promoter holding leaves roughly ₹11,800 Cr of free float and about 1.4 lakh shares of daily volume. For a PMS building a position, entry and exit costs are real and position sizing should reflect that — not a business risk, but a portfolio-construction one.

WHAT I AM WATCHING

- **Q1 FY27 results (5 August 2026):** the first print with any TruBridge contribution is thin (the deal closed 9 July), but management commentary on integration milestones and on the FY27 margin trajectory is the most important disclosure of the year.
- **TruBridge organic revenue disclosure:** whether IKS reports the acquired business separately. If it does not, the market loses its ability to check the thesis, and I would discount accordingly.
- **First cross-sell wins into the hospital base:** even two or three named coding or documentation contracts inside TruBridge accounts would validate the US\$575mn whitespace claim.
- **Deleveraging pace:** net debt/EBITDA below 2.0x by Q4 FY27 would confirm the cash-generation case and open the door to me adding more.